

Haven Paediatric Centre

Strategic plan: the moves, costed

Fifteen concrete moves across revenue, cost and structure, each with what it is, what it earns or saves, what it takes, and when. Grounded in the Lagos market as it prices care today.

Date: 26 August 2026

For: Mr Kabir Aregbesola, Mrs Abisodun Alli, Dr Shakirah Saliu, Dr Odedina, Mr Ogochukwu Odum

From: Dr Debo Odulana, Founding Partner, Consult for Africa



A letter to the founders

Dear Kabir, Abisodun, Shakirah, Dr Odedina and Ogochukwu,

Thank you for the way you received the diagnostic audit. You read a hard document honestly and agreed it is a true picture of where Haven stands. That acceptance is the best predictor of a turnaround that works.

Our first outlook set the direction: a ladder of ambition and the levers to climb it. You told us, rightly, that it was not concrete enough. It named revenue as a lever without saying reprice the NICU, launch a membership, or move the pharmacy to consignment. This document fixes that. It is the plan itself: the specific moves, what each one is, what it earns or saves, what it takes, and when it happens. Where a number needed to be real rather than asserted, we went and found it, and where a figure genuinely cannot be known from a desk, we say so and name the call that closes it.

Read this as a menu with a recommended order, not a fixed contract. The moves are real, the numbers are grounded in the Lagos market as it prices care today, and every one of them is something we can start on within weeks. What remains yours is the choice of how many to take on at once, and how fast.

With respect and real optimism,

Dr Debo Odulana

Founding Partner, Consult for Africa

1. The plan on one page

Haven has the hard thing already: a staff and a culture that care, confirmed by your own survey. What it lacks are the systems that turn that care into a durable business. The audit found revenue captured on a nursing handover sheet, around N7M of working capital trapped in receivables and stock, drugs going out of stock, and a licence that has to be squared with what you now do. None of that is a failure of effort. It is the work of a young hospital that has not yet been professionalised.

The plan below is fifteen concrete moves in four groups. The logic that orders them is simple and strict:

- **Fix capture before you drive revenue.** You cannot bank money you cannot record. The billing system and the tariff come first.
- **Free the trapped cash, then spend it on growth.** Consignment inventory and receivables recovery release the N7M that funds everything else.
- **Build the recurring relationship.** The Haven Club and the home-care line turn one-off visits into an annuity, and turn a clinic families visit when sick into a paediatric club families belong to.
- **Keep the clinical core, rent the rest.** Own the care and the culture. Outsource inventory, the lab, and the back office to people who do them for a living.

Every figure that follows is a Lagos-market benchmark as of mid-2026, sourced during this work. Prices in naira are written with an N. Where the market does not publish a number, we mark it clearly and name the short exercise that will get it.

2. Group A. Fix the revenue model



A1. Build the Haven Club: a paediatric health and development club

What it is. Not an insurance-style access plan, and not a replacement for how families join. Registration stays and rises from N10,000 to N15,000, paid by everyone who opens a file. On top of it sits the Haven Club: an annual membership that turns Haven from a clinic a family visits when a child is sick into a paediatric health-and-development club a family belongs to. Membership is optional and it is genuine white space, since no Lagos paediatric hospital runs anything like it. A non-member still registers at N15,000 and pays per visit exactly as today; the club converts the willing into a recurring, high-value relationship rather than forcing everyone into one.

Why a club, not cover. Parents buy clinical care because they must. They renew a club because it removes fear, gives them a named person who knows their child, and makes them feel like a good, connected parent. Treating illness is the need; reassurance, belonging, child development and status are the wants, and the wants are what a parent pays a premium to keep. The evidence is strong. A paediatrician is the single most trusted adviser a parent has. Most new parents report loneliness and say they would value a way to connect with other parents, a need no HMO can meet. And in membership medicine the relationship, not the discount, is what drives renewal. So Haven should sell the club, not the cover, and the free hospital "kids clubs" that already build loyalty elsewhere show the model works; Haven simply makes it a paid, premium one.

What every member gets, the club itself. These run across every plan and are the emotional core of the product:

- A named paediatrician who knows your child, and the continuity of that relationship.
- A 24/7 direct nurse-and-doctor line, and priority, no-queue, unhurried appointments.
- A child growth-and-development record you can see, with milestone tracking.
- A member events calendar: developmental play sessions, parent workshops, child and parental wellbeing circles, and seasonal socials.
- A parent community, so members belong rather than merely attend.
- Milestone moments: a first-haircut ceremony with a keepsake, a milestone photo, member-only events.
- A yearly paediatric first-aid and CPR session for your nanny or caregiver.
- Guaranteed member discounts on everything that is billed, and on partner services.

The plans, by the child's stage. The mistake to avoid is a generic ladder where a parent pays more for perks their child will never use; a parent of a four-year-old does not need newborn or NICU priority. So each plan bundles the essentials, needs and wants of one stage, wrapped in the club.



Plan	For	Annual price	Essentials, needs and wants
First Year	Birth to 15 months	N600,000 standard; N1,200,000 all-branded vaccines	Essentials: the full immunisation schedule, all scheduled well-baby visits, unlimited sick consults with the fee waived, a newborn home check and jaundice screen. Needs: the new-parent survival bundle, lactation support, an infant-sleep session, a weaning workshop, developmental monitoring. Wants: a postpartum parent circle, a newborn milestone photo, baby-club events
Growing Child	1 to 5 years	N350,000 to N500,000	Essentials: unlimited sick consults with the fee waived, booster vaccines, an annual developmental check. Needs: quarterly growth and development monitoring, nutrition guidance, positive-parenting workshops. Wants: member developmental play sessions, discounted swim and enrichment, a first-haircut ceremony, birthday and social events
School Age	5 to 16 years	N180,000 to N280,000	Essentials: an annual full check with school and sports health forms, six sick consults with the fee waived, HPV and boosters at cost. Needs: growth and puberty check-ins, an emotional-wellbeing check. Wants: holiday activity access, member events, discounts on partner services

The N15,000 registration is embedded in every plan; a non-member still pays it to open a file and is billed per visit. Siblings are bundled at a family rate, each additional child at 60 to 70% of their plan. These prices are a starting position, to be finalised against the tariff mystery-shop and a short willingness-to-pay test with real parents.

Priced to the family wallet, not to an HMO. This is a discretionary lifestyle purchase, so it is anchored to what affluent Lagos parents already spend on their children, not to an insurance premium. A premium creche runs N450,000 to over N3,000,000 a year, and private school fees from N1,500,000 per child. Closer to home, Nigeria already has a proven premium health subscription: Reliance's Alafia Serenity sells at N342,000 to N931,000 a year on experience and access rather than a naira cap. Against those



anchors, a First Year club at N600,000 to N1,200,000, which folds in a vaccine schedule worth about N834,000, and a Growing Child club at N350,000 to N500,000, sit comfortably inside the wallet and read as good value rather than expensive cover. The willingness to pay is real: affluent Nigerian parents already pay privately, and often informally on top, to secure continuity, a trusted doctor and no queue, which is exactly what the club sells.

Start before birth. The moment a family's loyalty is set is during the pregnancy, not after it, so the club opens with a Bump-to-Baby on-ramp: antenatal and newborn-preparation classes, lactation and birth-readiness sessions, and a postpartum circle, sold to expectant parents even though Haven does not deliver babies. It captures the family before the baby arrives, at the point of highest anxiety and highest openness, and channels them straight into the First Year plan on the day they leave the delivery hospital. Premium expectant parents already pay well for this: standalone antenatal education in Lagos runs into the hundreds of thousands of naira, and the strongest single differentiator is postpartum maternal mental-health support, badly under-served for affluent mothers against a measured 36% rate of postnatal depression.

What the club covers, and what it still bills. The club must not drift into insurance. It covers what is mainly the team's time and the programme: the consultation, the well-child visits, the 24/7 line, the events and the development record, plus, where a plan says so, the vaccine schedule, which is the one consumable a plan carries and only because its price is built to. Everything with a real per-unit cost, diagnostics, medicines, admissions, procedures, imaging and the NICU, is still billed, at the guaranteed member discount rather than free. Waiving a consult fee is affordable because a consultation is capacity, not stock; waiving the tests, drugs and admissions on top would turn the club into unpriced insurance and drain it. So "unlimited consults" means unlimited access to the doctor for the child's own illnesses, on a teleconsult-first, fair-use basis, and whatever follows is charged at the member rate.

Make it a visible win, then engineer the renewal. Value has to be legible or parents will not renew, so publish the à la carte value of what is included next to each fee: on the Growing Child plan the unlimited waived sick consults alone, for the age that catches every bug going, clear the fee within a handful of visits, and the First Year plan folds the roughly N834,000 branded vaccine bill into one predictable price. Then build for cadence, because members who engage at least yearly renew far more often: the eleven well-child visits in the first thirty months, the events calendar and the milestone moments all manufacture reasons to come back, which is what turns a one-year sign-up into a ten-year relationship.

Own the moat, broker the rest. The services that make the club distinctive are the ones no one in premium Lagos supplies well today: lactation, infant sleep, weaning and nutrition, postpartum mental-health support, and paediatric first-aid, all clinically adjacent and all currently absent as named premium products. Haven should own these with its own clinicians; they are the moat. The lifestyle extras, swimming, music, enrichment classes, photography and soft-play, should be brokered through a curated partner directory at member rates, not built. Own what builds trust, rent what builds footfall.

What it takes. This is the move that most changes how Haven operates, and it is worth being honest about that. It needs a membership and community manager and an events calendar, space or partners for the classes and circles, the clinical hires or partnerships behind lactation, sleep and nutrition, a simple member app or record with a community group, and firm plan terms: enrolment tied to birth or a well-child visit, waiting periods on pre-existing conditions so the club does not just attract the already-sick, teleconsult-first triage, a non-refundable clause once vaccines are drawn, and a fall-back to EPI stock if branded vaccine supply fails. It is a real build. It is also what turns a paediatric clinic into a brand families stay loyal to for a decade.



Build it asset-light, and build for renewal. Two lessons from clubs that have run this model elsewhere. First, keep it light. A well-known London family members' club with a heavy premises collapsed because a capped membership could not carry the fit-out and staff, so Haven should lean on its clinical core and brokered partners rather than sink capital into a large physical club. Second, engineer the renewal deliberately: a twelve-month term, a published developmental cadence from the six-week check onward that self-generates visits, and named age stages a family graduates through. The stickiest thing the club sells is not the vaccines but the answer to "is my child alright, and am I doing enough," which is why the first year carries the most value and the most willingness to pay, and why the international models that actually recur, in India and the Gulf, are built on reassurance and access rather than on the jobs.

When. Design the club and the survival-bundle hires in months two to four; launch to every discharge and new registration from month four; layer the events calendar and partner directory across months four to nine.

When. Design in months one to two, sell from month three, at every discharge and every new registration.

A2. Review and reprice the tariff

What it is. A full repricing of the self-pay tariff against the Lagos market, which Haven has almost certainly never done deliberately.

The benchmarks. A paediatric consult in private Lagos clusters at N20,000 to N30,000. A live comparable price list (Optimal, Surulere) anchors the procedure lines: phototherapy N6,000 a night, nebulisation N3,500, circumcision N12,000 to N14,500, incubator and nursery care N10,000 a night, a one-to-two-day admission N100,000. Set these as the floor of the range, not the ceiling, given Haven's GRA Ikeja catchment.

The structural point about payers. Anchor every price on self-pay, then treat HMOs as a separate and much lower schedule to be negotiated line by line. The gap is not small. An HMO pays a facility roughly N11,000 to N17,000 for a child for a whole year under the current capitation, and its per-service floor is around N700, an order of magnitude below any real consult fee. Capitation never covers a NICU stay or an admission. So watch the payer mix deliberately: HMO volume fills beds but erodes margin, and the two must be balanced on purpose, not by drift.

What it takes. The one genuine gap is that named competitors do not publish room-and-board day rates anywhere. Close it with a short mystery-shopper exercise: five or six calls to comparable hospitals (Reddington and Duchess in Ikeja, Lagoon, First Consultants, R-Jolad) for consult, ward, private-room and NICU day rates. That is a week of work, not a research project, and we would run it.

When. Months one to two, alongside the billing system so the new prices are actually captured.

A3. Reprice and fill the NICU

What it is. Treat the three NICU cots as the highest-yield asset in the building and manage them as one. Two moves: reprice, and feed.

Reprice. Haven's own NICU currently runs at about N2M for a week, roughly N285,000 a day, and your founder has described it publicly as among the cheapest available. That is a stated invitation to leave money on the table. A stable neonatal week at a GRA facility with a real governance standard can carry more, and the market gives no published competitor day-rate to contradict a careful increase. The benchmarking calls in A2 set the ceiling.

Feed. The durable growth is a referral pipeline. Haven does not deliver babies, so its NICU depends on referrals from the maternity hospitals that do, and those hospitals are charging N350,000 to N3M for a



caesarean before any neonatal complication arrives. A structured set of referral relationships with mid-tier maternity units across Ikeja and the mainland, backed by the governance standard that makes them trust you with a sick newborn, is what turns three cots from occasional to full.

What it takes. The governance and team-readiness work in the professionalise stage has to come first; a referral partner is trusting you with the highest-stakes patient there is. Then a named clinical lead owns the referral relationships.

When. Reprice in months two to three; build the referral pipeline across months three to six.

A4. Renegotiate the HMO book and recover the receivables

What it is. Two linked jobs: reprice the HMO contracts to the new tariff logic, and clear the N4.2M sitting in HMO receivables, most of it in two names.

Why now. The audit found close to a full period's revenue locked in the HMO ledger. That is cash Haven has already earned and is lending to insurers for free. Recovering it, and installing the discipline that keeps it recovered, is faster money than any new patient.

What it takes. A receivables recovery push on the two large balances as a quick win in the first fortnight; a monthly reconciliation and submission discipline so claims are clean and paid on time; and a contracting round that resets each HMO's paediatric tariff and drops any payer that will not move off a loss-making rate. The capitation figures above are the reference the HMOs themselves now anchor to.

When. Recovery from week one; renegotiation across months two to four.

A5. Run the pharmacy as a managed retail line

What it is. Stop treating the pharmacy as a store cupboard and run it as a retail business, with a real attach rate on every encounter and a margin protected by the inventory change in C1.

The finding it fixes. The audit showed a pharmacy "profit" of 201% that is a markup, not a margin, and stock going out at the wrong times. Proper attach discipline, formulary management and the consignment model in C1 together turn the pharmacy from a cash trap into a contributor.

What it takes. This move depends on C1 (vendor-managed inventory) and on the billing system in C4. On its own it is a set of standards; with those two it is a margin line.

When. Months two to four, following the inventory change.

3. Group B. Build new revenue lines

B1. Launch a paediatric home-care service

What it is. A hospital-run home-care line, starting with the services that carry the least clinical risk and the most demand, and building toward one that no one else in Lagos offers.

The whitespace. There is no commercial home phototherapy service for neonatal jaundice in Lagos, despite the clinical evidence supporting it and despite jaundice being one of the commonest reasons a newborn comes back. That is a service Haven, as a paediatric specialist, is uniquely placed to own.

The line-up and pricing.



Service	Suggested price	Notes
Newborn day-3 and day-7 home check	N15,000 to N30,000	Weight, feeding, cord, jaundice screen; converts every discharge
Home sample collection	N8,000 to N15,000	Leverages your own lab; low risk
Post-discharge and post-NICU follow-up	N20,000 to N40,000	Reduces readmission; a retainer anchor
Home phototherapy	N60,000 to N120,000 per episode	The differentiator: biliblanket plus daily nurse review and bilirubin monitoring, with hard escalation criteria; never for severe, exchange-range jaundice
Home vaccination	N10,000 to N20,000 call-out	Convenience premium for affluent families

The economics. A Lagos registered nurse costs about N140,000 to N190,000 a month, a neonatal nurse more. At three to four visits a day the fully-loaded cost is roughly N2,000 to N2,600 a visit before transport, so the headline prices carry healthy contribution. The real cost driver is Lagos traffic, which is why visits must be clustered by area and distance gated with a travel surcharge. The line only makes money above about 65% nurse utilisation, which is exactly why it should be sold through the membership rather than purely on demand.

Proof the model travels. The subscription-home-visit model is already proven in Nigeria: an elderly-care operator runs it at over 30,000 subscribers, with an annual plan around N378,000 for twelve visits. Haven would replicate that shape for children.

What it takes. Home care is a distinct licensed category, so a separate registration with the Lagos regulator, which requires a mandatory ambulance, an emergency kit, and a registered nurse plus aides under a supervising doctor. Because Haven is the referring hospital, it satisfies the provider-to-hospital communication requirement natively and can escalate any home case straight into its own NICU, a safety story no standalone agency can match. The phototherapy device and its clinical protocol are the gating items for the marquee service.

When. Start visits one to three in months three to four from your own patient flow; pilot home phototherapy once the device and protocol are in place.

B2. Add a telehealth follow-up service

What it is. Scheduled virtual reviews for post-discharge, chronic and reassurance visits, bundled into the membership and sold standalone to non-members.

Why it fits. It monetises the nurse advice line the membership already promises, reduces avoidable clinic visits, and is the cheapest possible touchpoint to keep a family engaged between physical visits. It is the connective tissue between the membership, home care and the clinic.

What it takes. A simple booking and video tool, a triage protocol so a virtual visit escalates cleanly to a physical one, and a clear price both inside and outside the membership.

When. Months three to five, alongside the membership rollout.

B3. Make immunisation a marketed product



What it is. Turn vaccination from an incidental service into a named, marketed product line, both as the anchor of the membership and as a standalone draw.

Why now. Vaccines are the reason young families choose a paediatric relationship and stick to it, and the private schedule is a high-value line: a single PCV dose retails around N60,000, the six-in-one around N57,000. A hospital that is known, specifically, as the reliable place for a child's full schedule wins the whole family's paediatric spend for years.

What it takes. Reliable branded stock with cold-chain integrity, a published schedule and reminder system, and clear pricing that separates the free-EPI core from paid branded and optional vaccines. This is mostly discipline and marketing, not capital.

When. Months two to four, woven into the membership.

B4. Sign corporate and school retainers

What it is. Recurring paediatric-care contracts with schools and with employers who want a child-health benefit, starting from the existing Toddler Town relationship.

Why it fits. Schools and corporates buy in bulk and pay predictably. A school health retainer, a set of member families delivered in one contract, and an employer child-health benefit are all annuity revenue that fills quieter clinic hours and feeds the membership.

What it takes. A simple B2B offer sheet, a named person owning the relationships, and the membership product in place to sell into them.

When. Months four to six, once the membership and home care give you something concrete to sell.

4. Group C. Rebuild the cost and operating structure

C1. Move the pharmacy to vendor-managed inventory

What it is. Shift pharmacy and consumables from self-owned stock to a consignment model, in which the supplier owns the stock sitting on your shelves, you pay only when a drug is dispensed, and the supplier carries the expiry risk.

The finding it fixes. The audit found N2.77M of cash tied up in pharmacy stock, alongside stock-outs of essentials. Consignment frees that cash directly and ends the stock-outs, because keeping the shelf full becomes the vendor's problem, not yours.

The options. A national operator runs exactly this consignment model with a roughly one-month setup and claims meaningful price reductions at scale. A second, larger platform offers financed just-in-time procurement instead, keeping ownership and margin with you while guaranteeing fast Lagos delivery and verified, anti-counterfeit stock. A third provider offers a comparable hospital model, but it is a Consult for Africa client, so we name it only for completeness and would not steer you to it; the choice must be made on its own merits and fully disclosed.

What you must keep control of. The paediatric and NICU formulary, so the vendor stocks what a children's hospital needs and not just its standard catalogue; your own retail pricing; controlled drugs, which stay ring-fenced under your superintendent pharmacist as the law requires; a monthly reconciliation of consumed-versus-owned stock, since that is the classic dispute in these deals; and a second supplier for critical lines so you are never single-sourced into a stock-out. Before signing, make each provider quantify the working capital it will release against your N2.77M and the stock-out reduction it will deliver.

When. Months two to four.



C2. Outsource the laboratory to an accredited reference partner

What it is. Run a tight point-of-care core in-house and send everything else to one accredited reference lab on a white-labelled referral account, rather than building and staffing a full laboratory.

The split. Keep in-house only the tests where speed changes management within the hour: blood glucose, malaria rapid test, haemoglobin and basic blood count, urinalysis, C-reactive protein, blood grouping, and, most important for a paediatric hospital, a bedside bilirubin analyser for jaundice. Send out everything infrequent or specialised: extended chemistry, histopathology, cultures, hormonal assays and molecular tests.

Why outsource. It converts laboratory capital expenditure into a per-test operating cost, avoids the scarce-scientist staffing burden and the accreditation load of running your own lab, and still lets you capture the margin between the partner's wholesale price and your retail price. Accredited partners exist in Lagos with international-standard certification.

The integrity line, drawn deliberately. The market norm in Nigerian diagnostics is a 20% referral kickback, funded by inflating the patient's test bill. It has been documented across the major labs, and at least one refused to take part, calling it a sharp practice. Haven should reject it outright. Given that we have built this whole engagement on integrity, the recommendation is to capture lab margin the clean way, a transparent negotiated wholesale-to-retail spread with white-labelled reporting, and never a hidden markup passed back as a kickback. It is the more defensible position and, over time, the more durable one.

What it takes. A short request for quotation to two or three accredited partners for wholesale pricing, turnaround times, pickup frequency and white-label reporting, and a written policy that fixes the point-of-care core and the send-out list.

When. Months two to four.

C3. Outsource the rest of the non-core

What it is. Hand the functions that are not Haven's craft to people who do them for a living: payroll, biomedical-equipment maintenance, security, laundry, and parts of finance.

Why it fits the thesis. The founders' time and the hospital's cash should sit on clinical care, culture and growth. Every hour spent supervising a laundry or a generator is an hour not spent on the things only Haven can do. Outsourcing these converts fixed headaches into predictable service contracts.

What it takes. A make-versus-buy pass over the cost base, then clean service-level agreements for each function handed out.

When. Months three to six, in parallel with the senior operations hire who will manage the contracts.

C4. Install a real revenue-capture and billing system

What it is. Replace the nursing handover sheet that currently serves as the revenue record with a proper capture and billing point, so every service delivered is recorded, priced and billed.

Why it is first among equals. This is the foundation the entire revenue side stands on. The membership, the repriced tariff, the pharmacy attach and the NICU repricing all assume that what Haven does is what Haven bills. Today it is not, and that gap is pure lost revenue. Nothing else in Group A pays out reliably until this is fixed.

What it takes. A billing and light patient-record system suited to a facility this size, configured to the new tariff, with the reporting layer the audit found missing, so the numbers finally reconcile and the board can



see the business at a glance.

When. Months one to two. This is the first build.

5. Group D. Set the structure straight

D1. Resolve the licence

What it is. Square the establishment licence with the service Haven actually delivers, and resolve the licensing question the audit raised, before the facility scales further into complex neonatal care.

Why it gates everything. A licence that does not match the service is a risk that grows with every step up the ladder. It has to be clean before Haven markets a NICU referral pipeline, an accreditation, or a second site.

When. Months one to two, as part of stabilising.

D2. Bring in a senior operations leader

What it is. Put an experienced operations leader in post to run the facility day to day, with the role and its measures defined first so the hire lands into clarity.

Why it is the keystone. Almost every move in this plan needs an owner who is not one of the founders. The audit's plainest finding was the absence of a senior operator; that is the person who runs the billing system, manages the vendor contracts, holds the referral relationships and reports the numbers honestly. Without this hire the plan depends on the founders' own bandwidth, which is exactly the constraint the plan is meant to remove.

What it takes. Define the role and its measures during the early work, then recruit into it. Consult for Africa can source and vet the person through our workforce platform, including senior operator and diaspora profiles.

When. Define in months one to two; in post by month three.

6. Sequencing and what it takes

The moves are ordered so that the early ones fund the later ones. The first window is mostly plumbing and cash release; it costs little and pays for the rest.

Window	The moves	What it is buying
Months 1 to 2	Billing system (C4), tariff review and mystery-shop (A2), licence (D1), define the operations role (D2), receivables recovery (A4)	Capture, price, cash, and legal ground
Months 2 to 4	Vendor-managed inventory (C1), lab outsourcing (C2), operations leader in post (D2), NICU reprice (A3), pharmacy line (A5), HMO renegotiation (A4)	Freed cash, lower cost, a professional core
Months 3 to 6	Membership launch (A1), home care (B1), telehealth (B2), immunisation product (B3), NICU referral pipeline (A3)	The recurring revenue engine



Window	The moves	What it is buying
Months 4 to 6	Corporate and school retainers (B4), outsource remaining non-core (C3)	Annuity revenue and a lean back office

The investment logic. The two moves that cost real money up front are the billing system and, if you choose it, the home-phototherapy capability. Almost everything else is either cash-releasing (consignment inventory, receivables recovery), margin-improving at little cost (tariff, pharmacy, lab), or self-funding once live (membership, home care). The plan is designed so that the trapped N7M and the early margin gains pay for the growth moves, rather than asking the founders for a large new injection.

What it asks of you. Money is the smallest ask. The larger ones are control, letting a senior operator run the place, and focus, being willing to rent the lab, the pharmacy logistics and the back office rather than own them. Those are the real trade-offs, and they are exactly what the founders' survey is designed to surface.

7. A note on the survey

This plan is paired with a short survey sent to each founder individually. It does not ask whether you support any of these moves, because you would all say yes and we would learn nothing. It forces trade-offs instead: it asks you to divide a fixed pool of investment across these priorities so we see what you would actually fund first, it puts the real tensions as direct either-or choices, and it asks what you believe your fellow founders think. The gap between your own view and your assumptions about each other is where the alignment work is, and it is best surfaced before the money is committed. Please complete it alone, before conferring.

8. Basis and caveats

Every naira figure here is a Lagos-market benchmark gathered during this work, current to mid-2026. Two honest limits. First, named Lagos hospitals do not publish room, ward or NICU day rates anywhere, so those specific benchmarks require the short mystery-shopper call-around described in A2, which we would run. Second, the exact commercial terms of the inventory and laboratory partners are not public and must be settled by direct quotation; the models and the ranges are sound, the final rates are a negotiation. Where a number could not be verified, we have said so rather than invent it. That discipline is the same standard we are asking Haven to run itself by.

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